



**FEMINISTS FOR PEACE, RIGHTS AND
JUSTICE CENTRE**

Advocacy, Child Protection & Safeguarding Toolkit

**A Practical Resource for community based child safeguarding
advocacy and policy engagement to prevent SGBV and related
violence in learning institutions and in the community**



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About FPRJC

Foreword

Feminists for Peace, Rights and Justice Centre (FPRJC) is a grassroots, survivor-led feminist organization working in Kibera to advance gender justice, human rights and the holistic wellbeing of women and girls in all their diversities. Our mandate is rooted in strengthening dignity, agency and resilience through integrated programming across women's health, rights and wellbeing, feminist leadership and governance and climate justice and livelihood development. We center the lived experiences of women, girls, and marginalized communities, working to dismantle systemic inequalities and create pathways for safer, more just, and inclusive societies.

Young girls in Kenya continue to face complex and overlapping challenges; including exposure to sexual and gender-based violence (SGBV), online exploitation, limited access to justice and inadequate support systems. These realities call for intentional, coordinated and community-driven responses that prioritize prevention, protection and empowerment.

This Advocacy and Safeguarding Toolkit has been developed as a practical resource to equip community-based organizations, youth leaders, and frontline advocates with the knowledge and skills needed to engage effectively in advocacy, prevent harm and respond to cases of abuse in safe, ethical and survivor-centered ways. The toolkit is grounded in feminist, rights-based and inclusive approaches that ensure no one is left behind, particularly girls and children with disabilities.

We recognize that sustainable change begins within communities. By providing simplified, evidence-informed tools and guidance, this manual seeks to strengthen grassroots advocacy, enhance safeguarding practices, and promote accountability among duty bearers. It also aims to foster collaboration between communities, civil society, and government actors to ensure that prevention and response systems are accessible, responsive, and effective.

We encourage all users to apply these tools responsibly and to continue championing safe, inclusive, and just communities for all.

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PURPOSE, SCOPE AND CONTEXT

Communities across Kenya continue to face significant challenges related to Sexual Gender Based Violence, child abuse and safeguarding gaps, particularly among girls and vulnerable populations. While protective policies and legal frameworks exist, a persistent gap remains at the community level in awareness, capacity and access to practical tools for prevention, response, and reporting.

In informal settlements like Kibra Sub-County, compounded marginalisation exposes adolescent girls to increased risks of violence both in and around learning institutions, as well as within homes and community spaces. Many girls and caregivers have little to no knowledge of their rights or access to clear and safe referral pathways, limiting their ability to seek timely support and protection.

Community-based advocates; including youth leaders, community health volunteers, promoters, teachers, school-based mentors and grassroots organisations, are frequently the first point of contact for survivors. However, many lack access to simplified, user-friendly guidance on how to:

- Plan and implement advocacy initiatives for safer school and community environments
- Deliver accurate and sensitive messaging on SGBV, child protection, and safeguarding
- Engage safely, ethically, and confidently with survivors, particularly adolescent girls
- Navigate reporting and referral systems within school and community structures
- Engage with policymakers and duty bearers to strengthen accountability and protection systems

The increasing use of digital platforms and social media has also introduced new risks, including online exploitation, harassment and abuse, particularly affecting girls and young people, and requiring updated, context-specific safeguarding responses.

This toolkit was developed to address these gaps by providing a comprehensive yet accessible resource tailored to the needs of community-based advocates working in and around learning institutions. Its purpose is to strengthen advocacy, child protection, and safeguarding practices by equipping frontline actors with practical knowledge, tools, and step-by-step guidance to prevent violence, respond safely to disclosures, and support girls in accessing appropriate services and protection.

The toolkit is designed for use in both real-time response and capacity strengthening. It can support immediate action when handling cases, while also serving as a guide for training, planning, and strengthening community and school-based systems. Users are encouraged to apply the guidance step-by-step where possible, while adapting to the realities of each situation, always prioritizing the safety, dignity, and best interests of the child. It should be used alongside existing referral systems and does not replace professional medical, legal, or psychosocial services, but rather supports safe engagement and effective linkage to care.

All actions under this toolkit are guided by principles that reflect feminist values, ethical standards, and the lived realities of girls in and around learning institutions in Kibra.

GUIDING PRINCIPLES

Safety and dignity must be prioritised at all times

A *do no harm* approach must be centered in every engagement. This means assessing risks before taking action, ensuring that responses do not expose survivors to further harm, stigma or retaliation, and always upholding the dignity and wellbeing of girls and survivors.

Inclusivity and intersectionality are central to response

Experiences of violence and access to support are shaped by gender, age, disability, sexual orientation, and social context. Responses must be inclusive, non-discriminatory and sensitive to the diverse realities of those affected.

Confidentiality must be maintained consistently

Information shared by survivors should be treated with the highest level of confidentiality. It should only be shared with relevant service providers and only when necessary to support the survivor, ensuring their safety and trust are not compromised.

Participation strengthens ownership and effectiveness

Community members, including girls and young people, should be meaningfully engaged in identifying risks, shaping solutions and strengthening protective systems. Participation builds trust, relevance and sustainability of interventions.

Accountability is essential for protection and justice

Advocacy and response efforts should strengthen reporting and referral systems, while ensuring duty bearers, including schools, community leaders and institutions, are held responsible for prevention, protection, and response to violence.

All engagement should be survivor-centred and ethical

Survivors should be treated with respect, empathy and without judgement. Their experiences, choices and pace must guide all actions, recognizing their agency while prioritizing their safety and best interests.



Acronyms and Key Definitions

Acronyms

Acronym	Meaning
SGBV	Sexual and Gender-Based Violence
SRHR	Sexual and Reproductive Health and Rights
NGO	Non-Governmental Organisation
CBO	Community-Based Organisation
CHP	Community Health promoters
PWD	Persons with Disabilities

Key Definitions

Advocacy

Speaking up and taking deliberate action to influence decisions that affect people's lives.

Safeguarding

Measures put in place to protect individuals, especially children and vulnerable persons, from harm, abuse, and exploitation.

Survivor

A person who has experienced violence or abuse. This term is preferred over 'victim' as it affirms strength and dignity.

Child Protection

Actions taken to prevent and respond to violence, exploitation, and abuse against children.

Duty Bearers

Individuals or institutions responsible for upholding and delivering rights (e.g., government, police, health workers).

Rights Holders

Individuals entitled to rights; including children, adolescents, and community



Chapter One: Understanding Advocacy



1.1 Introduction

Advocacy is a powerful tool that enables individuals, communities, and organisations to speak up, influence decisions, and create positive change. For community-based organisations (CBOs), non-governmental organisations (NGOs), and local advocates across Kenya, advocacy plays a central role in addressing issues such as SGBV, child protection, education, health, and social justice. This chapter introduces the concept of advocacy and provides practical, foundational guidance for community-based advocates to understand and begin engaging in advocacy work.

1.2 What is Advocacy?

Advocacy means speaking up and taking action to influence decisions that affect people's lives. It involves:

- Raising awareness about an issue
- Influencing policies, laws, or practices
- Promoting the rights of individuals or communities
- Holding leaders and institutions accountable

In simple terms: Advocacy is about creating change by using your voice and working with others.

1.3 Why is Advocacy Important?

Advocacy is important because it helps to:

- Protect rights, especially for children, adolescents, marginalised and vulnerable groups
- Improve access to services such as health, education, and protection
- Influence policies and laws to better respond to community needs
- Promote accountability, ensuring leaders and institutions fulfil their duties
- Empower communities, giving people the confidence to speak up and act

CASE STUDY

Advocacy in Action

Community advocacy in Kenya has contributed to improved access to free primary education, strengthened child protection systems, and increased awareness of gender-based violence reporting mechanisms. These changes demonstrate what is possible when communities organise and speak with a united voice.

1.4 Types of Advocacy

This focuses on speaking up for yourself or supporting an individual to access services, protection or justice. It involves walking with survivors through systems and ensuring their rights are upheld.

Individual Advocacy

This focuses on speaking up for yourself or supporting an individual to access services, protection or justice. It involves walking with survivors through systems and ensuring their rights are upheld.

Example

Supporting a girl who has experienced violence to report a case, access medical care and navigate referral pathways safely.

Community Advocacy

This involves working collectively with community members to address shared challenges and strengthen local protection systems. It aims to shift norms, build awareness and promote collective responsibility.

Example

Organising a community dialogue or baraza to discuss child protection, safeguarding and the role of schools and families in preventing violence.

Example

Organising a community baraza to discuss child protection and safeguarding responsibilities.

Policy Advocacy

This focuses on engaging duty bearers, leaders and decision-makers to influence policies, systems, and institutional practices. It seeks to strengthen accountability and ensure existing laws and frameworks are effectively implemented.

Example

Engaging county officials or education stakeholders to improve enforcement of child protection and safeguarding policies in schools at community level..

Example

Advocating for improved implementation of child protection policies at the county level.

Media Advocacy

This uses media platforms, both traditional and digital, to raise awareness, influence attitudes, and amplify community voices. It is particularly useful for reaching wider audiences and shaping public discourse.

Example

Sharing SGBV prevention messages through local radio stations or community notice boards to promote safer environments for girls.

1.5 Principles of Effective Advocacy

For advocacy to be both effective and ethical, it must be guided by the following core principles:

Principle	What it means in practice
Rights-Based Approach	All advocacy must promote and protect the human rights of every person, especially those who are vulnerable.
Meaningful Participation	Communities must be actively involved in identifying issues and shaping solutions. Nothing for us without us.
Evidence-Based	Use facts, data, and lived experience to support your messages and strengthen your case.
Inclusivity	Ensure the active participation of girls, boys, young people, persons with disabilities, and marginalised groups including intersex persons and LGBTQI+ individuals.
Accountability	Advocates should promote transparency and hold duty bearers responsible for their commitments.
Do No Harm	Advocacy must never expose individuals, especially survivors, to further harm, stigma, or risk.

1.7 The Advocacy Cycle

Advocacy is not a one-time activity, it is an ongoing, cyclical process. The following steps, explored in detail in Chapter Two, form the foundation of any well-structured advocacy plan:

#	Stage	Key Question
1	Identify the Issue	What problem are you trying to solve?
2	Set Objectives	What specific change do you want to see?
3	Identify Target Audience	Who has the power to make this change?
4	Develop Key Messages	What do you want to communicate?
5	Choose Strategies	How will you deliver your message?
6	Implement the Plan	Carry out your planned advocacy activities.
7	Monitor and Evaluate	What worked? What needs to be improved?

1.8 The Role of Community-Based Advocates

- Community advocates play a critical and often irreplaceable role in the protection of children and vulnerable individuals. Their responsibilities include:
- Identifying and reporting cases of SGBV and child abuse
- Raising community awareness on prevention and protection
- Linking survivors to appropriate services, including health, legal, and psychosocial support
- Engaging local leaders such as chiefs, village elders, and county officials

REFLECTION ACTIVITY

What is Advocacy?

Ask participants the following questions and encourage them to draw on real-life experiences:

1. What issues affect girls in your community?
2. Have you ever spoken up about a problem? What happened?
3. Who are the key decision-makers in your community?

Facilitator Note

Create a safe and open space for participants to share their experiences. Personal examples help build a deeper understanding of advocacy in practice.

Key Takeaways

- Advocacy is about creating positive, lasting change.
- Everyone has the potential to be an advocate.
- Effective advocacy requires careful planning, credible evidence, and collaboration.
- Community voices carry real power in influencing decisions.
- Advocacy must always be safe, inclusive, and respectful.





Chapter Two: Step-by-Step Advocacy Planning

2.1 Introduction

Effective advocacy is the result of careful, structured planning. This chapter provides a clear, step by-step guide that community-based advocates can use to plan and implement advocacy activities on issues such as SGBV prevention, child protection, and adolescent wellbeing. The steps are practical, accessible, and applicable to a wide range of community advocates including community health volunteers, youth groups, teachers, social workers, and NGO/CBO staff.

2.2 The Advocacy Planning Cycle

Advocacy follows a continuous, iterative cycle comprising seven stages. Each stage builds on the previous one and informs ongoing learning and improvement.

Step 1: Identify the Issue

- Engage community members in conversations about the problems they face.
- Observe recurring challenges in the local environment.
- Review available local data, reports, and case records.
- Identify who is most affected and in what ways.

Ask yourself

What is happening? Who is affected? Where and why is it occurring?

Step 2: Set Advocacy Objectives

- Define the specific, measurable change you want to achieve.
- Ensure objectives are SMART: Specific, Measurable, Achievable, Relevant, and Time bound.
- Weak example: 'Reduce GBV in the community.'
- Strong example: 'Within 6 months, increase reporting of GBV cases by 30% through community awareness and referral support.'

Step 3: Identify Your Target Audience

- Primary targets (decision-makers): county officials, police, school administrators and teachers.
- Secondary targets (influencers): parents, religious leaders, youth leaders, and media.
- Focus on people who hold authority, can influence decisions, and are willing to engage.

Step 4: Develop Key Messages

- Keep messages simple, clear, and easy to understand.
- Ground messages in evidence and real community experiences.
- Make messages action-oriented and relevant to your target audience.
- Adapt the framing: leaders need policy-focused messages; communities need awareness and behaviour change messages.

Examples

Protect our children – report abuse early.' | 'Silence hides violence. Speak up and seek help.'

Step 5: Choose Your Strategies and Activities

- Community engagement: barazas, school talks, youth forums, home visits.
- Media engagement: local radio discussions, social media campaigns, posters, and flyers.
- Stakeholder meetings: engagements with chiefs, police, county officials, and policy dialogues.
- Awareness campaigns: roadshows, community events, and school outreach programmes.

Step 6: Implement the Advocacy Plan

- Assign clear roles and responsibilities to all team members.
- Set realistic timelines and milestones.
- Use available resources effectively and work collaboratively.
- Start with small, manageable actions and build momentum progressively.

Step 7: Monitor and Evaluate

- Track whether your objectives have been achieved.
- Indicators to monitor: number of people reached, increase in reported cases, policy or behaviour changes, community feedback.
- Reflect on what worked well, what challenges arose, and what can be improved.
- Use learning from each cycle to inform future planning.

2.3 Advocacy Planning Template

The template below can be used during advocacy planning sessions to structure your work clearly and systematically:

Planning Stage	Key Question	Your Response / Notes
Issue Identification	What is the problem?	
Objective Setting	What change do you want to achieve?	
Target Audience	Who can make this change happen?	
Key Messages	What do you want to say?	
Activities	How will you deliver your message?	
Timeline	When will activities take place?	
Monitoring	How will you measure Success?	

CASE STUDY

Increasing Reporting of Child Abuse — A Youth Group's Journey

A youth group in a rural community identified that cases of child abuse were going unreported due to widespread fear and social stigma. Using the advocacy planning cycle, they took the following steps:

- Identified the issue through structured community discussions and observations
- Set a clear objective: to increase the number of reported abuse cases within three months. Engaged chiefs, teachers, and community health volunteers as key stakeholders.
- Developed simple, powerful messages such as: 'Reporting abuse protects children.'
- Conducted school awareness sessions, community barazas, and radio talk shows.

Results

The initiative led to increased community awareness, a measurable rise in reported cases, and strengthened collaboration with local leaders and authorities.

REFLECTION ACTIVITY

Plan Your Own Advocacy

In small groups, ask participants to work through the following exercise:

4. Identify a community issue that matters to your group.
5. Develop one clear and SMART advocacy objective.
6. Identify one key target audience.
7. Craft one key message tailored to that audience.

Each group should present their plan to the wider group for discussion and feedback.

- Effective advocacy requires clear structure and intentional planning.
- Begin with a well-defined issue and a realistic, measurable objective.
- Identify the right people to influence and tailor your messages accordingly.
- Use simple, evidence-based, and compelling messages.
- Monitor your progress continuously and learn from each experience.



Chapter Three: Key Messages on SGBV Prevention and Child Protection



3.1 Introduction

Sexual and gender-based violence (SGBV) and child abuse are serious issues affecting many communities across Kenya. Community-based advocates play a vital role in raising awareness, preventing harm, and supporting survivors. This chapter provides clear, evidence-based messages that can be adapted for use in community meetings (barazas), school sessions, youth forums, home visits, and media engagements. All messages are designed to be easy to understand, culturally appropriate, age-sensitive, and action-oriented.

3.2 Understanding SGBV and Child Protection

Key Question	Key Question
What is SGBV?	SGBV refers to any harmful act directed at a person based on their gender. It includes: sexual violence (rape, defilement); physical violence; emotional and psychological abuse; early and forced marriage; and online sexual exploitation.
What is Child Abuse?	Child abuse includes: physical abuse; sexual abuse (defilement and exploitation); emotional abuse; and neglect.

Core Message

Every child has the right to be safe, protected, and treated with dignity.

3.3 Core Prevention Messages

The following messages are suitable for use across a range of settings and audiences:

Safety and Protection

- Every girl has the right to live free from violence, abuse and exploitation.
- Safe communities are built when individuals actively prevent, report, and respond to harm. Increased awareness of risks and rights contributes to reduced exposure to violence in homes, schools, and public spaces.

Speaking Up

- Silence hides violence, breaking silence enables access to support, protection, and justice.
- Survivors and witnesses who report cases are more likely to receive timely assistance and prevent further harm.
- Creating a culture of speaking up strengthens community accountability and protection systems. If you see something, report it.

Early Reporting

- Early reporting increases access to medical care, psychosocial support, and legal protection.
- Timely action improves case outcomes and reduces immediate and long-term harm to survivors.
- Clear knowledge of referral pathways ensures faster and safer responses to incidents.

Respect and Consent

- Understanding consent reduces incidents of sexual and gender-based violence.
- Respecting personal boundaries promotes safer relationships in schools and communities.
- Challenging harmful norms contributes to long-term behaviour change and gender equality.
- No means no, consent must always be given freely and without pressure.
- When dealing with children, always put the best interest of the child first.

Online Safety

- Practicing safe online behaviour reduces exposure to exploitation, harassment, and abuse. Not everyone online is who they claim to be.
- Increased digital awareness helps girls identify risks and protect their personal information.
- Safe use of digital platforms strengthens participation while minimizing harm. Do not share personal information or arrange to meet strangers alone.

3.4 Age-Appropriate Messaging

Audience	Recommended Messages
Ages 10–14 (Younger Adolescents)	Your body belongs to you. Say NO to anything that makes you feel unsafe or uncomfortable. Tell a trusted adult if someone hurts or threatens you.
Ages 15–19 (Older Adolescents)	Make safe choices both online and offline. Never meet someone you know only from online platforms without informing a trusted adult. Support your peers to seek help if they are in danger.
Parents and Caregivers	Talk openly with your children about personal safety and boundaries. Monitor your child's online activities and friendships. Create a home environment where children feel safe to report abuse. Believe and support children when they speak up.
Community Leaders	Protecting children is a collective responsibility. Take immediate action when cases of abuse are reported. Support the establishment of safe reporting systems. Promote a zero-tolerance stance on violence.

3.5 Do's and Don'ts When Delivering Messages

DO	DO NOT
Use simple, clear, and respectful language	Do not Blame or shame victims or survivors
Encourage open and honest discussion	Do not Use harsh, threatening, or judgemental language
Provide accurate, evidence-based information	Do not Share graphic or distressing details
Ensure confidentiality	Do not share any information that might compromise survivor's safety with anyone you do not have to
Promote hope, support, and available services	Do not Ignore cultural sensitivities or community context

3.6 Reporting and Referral Pathways

Advocates should always link their messages to clear information about where help can be found. Key contacts include:

Support/ Service Provider	Location	Phone Number
Child protection Services (Children's office)	Kenya Children Helpline	116
Gender Violence Recovery Centre	Kenya GBV helpline	1195
FIDA	Kenya	0800720501
Ambulance	Nairobi county	1508
Kibera Police Station Gender Desk	Kibera DC	
Jamhuri Police Station Gender Desk	Jamhuri	0781277997
Coptic Mission Hospital	Ngong road	0711043000

Support/ Service Provider	Location	Phone Number
Nairobi Women's Hospital GVRC	Adams/ Hurligham	0719638006/0709667000/ 0800720565
Mbagathi Hospital	Golfcourse, Ngumo	0202724712
Kenyatta Hospital	GBV Helpline	0800721038
Feminist for Peace, Rights and Justice Centre	Kibera	0757840731
CREAW	Kibera	0800720186
SHOFCO	Gatwekera	0737333741
Kibera Pride Children's	Olympic	info@kiberaprideinitiative.org

Always remind your audience: You are not alone... help is available.

CASE STUDY

Online Exploitation: A Cautionary Account

A 16-year-old girl was contacted via social media by an unknown individual who offered her what appeared to be a modelling opportunity. She was invited to meet him alone at a hotel. Without informing any trusted adult, she went unaccompanied and was subsequently abused.

Lessons Learned:

Not all online opportunities are genuine. Always inform a trusted adult before meeting someone you have only encountered online. Never meet a stranger alone.

CASE STUDY 2

School-Based Violence:

A 15-year-old girl began experiencing repeated harassment from a male classmate who would make inappropriate comments, touch her without consent, and threaten to spread rumours about her if she spoke out. Over time, the behaviour escalated to physical intimidation and emotional abuse, making her feel unsafe and anxious in school.

Fearing stigma and not being believed, she remained silent and her academic performance began to decline. Eventually, a teacher noticed her withdrawal and connected her to a school counsellor. With support, the case was reported and the school initiated disciplinary action while linking her to additional psychosocial support services.

Message:

Your safety is more important than any opportunity.

REFLECTION ACTIVITY

Reflection Activity: Message Practice

Ask participants to:

8. Select one message from this chapter.
9. Practice delivering it to the group as if speaking to a real audience.
10. Discuss as a group: How could the message be strengthened or adapted?

Key Takeaways

- Clear, simple messages are the foundation of effective community advocacy.
- Prevention begins with awareness, reach people before harm occurs.
- Communities must work together to protect children and vulnerable individuals.
- Survivors need support, understanding, and access to services, not blame.
- Always link your messages to concrete information about where to get help.

DO'S



I believe you

How can I support you?

It is not your fault

Your feelings are valid

KIBERA
TUNAWEZA
PAMOJA

WE LISTEN
WE BELIEVE
WE SUPPORT
WE RESPECT



**DO: Listen. Believe. Support.
Respect. Empower.**

DON'T'S



Why didn't you leave?

Are you sure?

It wasn't that bad

You should just move on



**DON'T: Judge. Blame. Dismiss.
Pressure. Minimize.**

Chapter Four: Do's and Don'ts of Survivor Engagement



4.1 Introduction

Survivors of SGBV and child abuse often carry deep emotional, physical, and psychological wounds. The way an advocate responds to a survivor can either support their healing and recovery or cause further harm. This chapter provides clear, practical guidance on how to respond safely and respectfully, communicate in a supportive and non-judgemental manner, and avoid actions that may retraumatise or endanger survivors.

Guiding Principle:

Always prioritise the survivor's safety, dignity, and well-being above all else.

4.2 Guiding Principles for Survivor Engagement

Principle	What it means
Safety First	Ensure survivors safety and wellbeing by assessing immediate risks, ensure they get emergency services including psychological first aid.
Confidentiality	Keep all information private and secure. Do not share without the survivor's informed consent.
Respect	Treat every survivor with dignity, empathy, and without judgement, regardless of their circumstances.
Non-Discrimination	Provide equal support to all survivors regardless of gender, age, disability, or background.
Informed Consent	Always seek permission from parent/ guardian before asking questions, sharing information, or making referrals. (Keep the child's best interest in mind)
Do No Harm	Avoid any action that may cause emotional distress or expose the survivor to stigma or retaliation.

4.3 Do's: Good Practices When Supporting Survivors

DO	DO NOT
Listen actively, give your full attention and allow the survivor to speak freely at their own pace without interruption.	Do not Blame or question the survivor: 'Why did you go there?' or 'Why didn't you report earlier?' Why were you abused?
Assess the survivor's physical and emotional state, if they are still in danger, and offer psychological first aid.	Do not put the survivor in any more danger.
Believe the survivor and take their account seriously without questioning their credibility.	Do not Force or pressure the survivor to share details or speak before they are ready.
Provide emotional support; show empathy, reassurance, and remind them they are not alone.	Do not Make promises you cannot keep, such as 'Everything will be fine' or 'I will solve this.'
Ensure privacy, speak in a safe, private space and protect the survivor's identity at all times.	Do not Share the survivor's information without their explicit consent.
Offer information and options, explain available services and allow the survivor to make their own choices.	Do not Judge or criticise the survivor based on moral, cultural, or personal views.
Refer to appropriate professional services — health facilities, police, counselling, and child protection services.	Do not Attempt to investigate the case yourself — leave this to trained professionals.
Follow up where appropriate to ensure the survivor has accessed the support they need.	Do not Delay referral — timely access to medical care (within 72 hours for sexual violence) is critical.

4.4 Step-by-Step Response Guide

When someone reports abuse or discloses a difficult experience, follow this structures;

response:

Step 1: Look

- Assess if the survivor is still in immediate danger and try to prevent further harm
- Assess the survivor's physical state
- Assess survivor's emotional state and offer PFA where needed

Step 2: Listen

- Stay calm and composed.
- Create a safe, private space for the conversation.
- Allow the survivor to speak without interruption or hurry.

Step 3: Comfort and Reassure

- Use supportive, non-judgemental language.
- Remind them: 'This is not your fault.' and 'You are not alone.'

Step 4: Assess Immediate Safety

- Determine whether the survivor is in immediate danger.
- Take necessary steps to ensure their physical safety.

Step 5: Inform

- Clearly explain the available support options; health, legal, counselling.
- Allow the survivor to make informed decisions about next steps.

Step 6: Link/Refer

- Connect the survivor to the appropriate professional services without delay.
- Ensure they understand the referral process and feel supported through it.

Step-by-Step Response Procedure

Step 1: Receiving a Case

- Create a safe and private space.
- Listen without interrupting and allow the survivor to speak at their own pace.
- Reassure them that they are safe and supported.
- *Do say: "I believe you" | "This is not your fault"*
- *Avoid blame or judgment.*

Step 2: Assess Immediate Safety

- Check if the survivor is in danger.
- If unsafe: move them to a safe space and seek support
- If safe: proceed to next step
- *Do not assume home is safe.*

Step 3: Obtain Consent

- Explain available options and let the survivor decide what to do.
- Respect their choices, including not reporting.

Step 4: Medical Response

- Refer to a health facility for treatment and documentation
- If later, still link to medical care
- Early care is critical for health and evidence.

Step 5: Psychosocial Support

- Provide reassurance and emotional support.
- Validate feelings and refer to counselling or safe spaces.
- Avoid forcing detailed recounting.

Step 6: Safe Referral and Reporting

- Link survivors to trusted service providers.
- Avoid unsafe or informal systems.
- Support reporting if the survivor chooses
- Continue support even if they decline

Step 7: Responder Safety

- Do not put yourself at risk.
- Avoid confronting perpetrators
- Work with trusted partners
- Keep cases confidential

Step 8: Safety Planning

- Help the survivor reduce future risk.
- Identify safe places and trusted people
- Avoid high-risk areas
- Agree on emergency contacts or code words

Step 9: Documentation

- Record only essential information and store it securely.
- Do not share identifying details without consent
- Protect survivor confidentiality at all times

Step 10: Follow-Up

- Check in within 24-48 hours and continue support as needed.
- *Be consistent but not intrusive.*

4.5 What to Say and What to Avoid

Supportive Statements	Statements to Avoid
What happened?/ 'I believe you.	'Are you sure that is what happened?'
'This is not your fault.'	'You should have known better.'
'You are not alone, support is available.'	'Why didn't you stop it earlier?'
'Help is available, and I can assist you to access it.'	'This will ruin your reputation.'

4.6 Special Considerations

Drug facilitated assault

Drug-facilitated assault is an emerging and often underreported form of violence, particularly in social and school-adjacent settings. Survivors may experience memory gaps, confusion, dizziness, or loss of consciousness, making it difficult to recall events or provide detailed accounts. This can increase fear, self-doubt, and delayed reporting.

- Treat all disclosures seriously, even where details are unclear or incomplete.
- Prioritise urgent medical attention, especially within 72 hours, for both health care and evidence preservation
- Avoid blaming or questioning the survivor's memory gaps.
- Support safe accompaniment to a trusted health facility and ensure confidentiality.
- Be aware that perpetrators may be known individuals, increasing risk of intimidation or retaliation

Children

- Use simple, age-appropriate language that the child can understand
- Involve a trusted adult throughout the process (where safe and appropriate)
- Follow established child protection procedures at all times
- Create a safe and non-threatening environment to encourage disclosure

Persons with Disabilities

- Ensure physical, communication, and information accessibility
- Be patient, respectful, and responsive to individual needs
- Recognise increased vulnerability to violence and barriers to reporting
- Where needed, involve support persons or assistive communication tools

Adolescents

- Respect their privacy and evolving autonomy
- Provide clear information and options to support informed decision-making
- Encourage safe and timely reporting without pressure
- Be mindful of peer dynamics, stigma, and fear of repercussions within school environments

CASE STUDY

An Unsafe Response — Learning from Mistakes

A 17-year-old girl disclosed an experience of abuse to a community volunteer. The volunteer responded by blaming her for meeting the perpetrator, shared the story with other community members without her consent, and delayed connecting her to professional support. The survivor felt deeply ashamed and refused to seek any further help.

Correct Approach:

Listen without judgement. Reassure and affirm. Maintain strict confidentiality. Refer immediately to the appropriate services.

REFLECTION ACTIVITY

Reflection Activity: Role Play

In pairs, participants practise survivor engagement using the following roles:

- One participant acts as the survivor.
- The other acts as the community advocate.

Focus on practising active listening, appropriate reassurance, and the referral process. After the role play, the group discusses: What went well? What could be improved?

Key Takeaways

- Survivors need support, understanding, and access to services, not judgement.
- Your response as an advocate can either support healing or cause further harm.
- Always prioritise safety, confidentiality, and respect in every interaction.
- Never attempt to investigate, refer survivors to trained professionals immediately.
- Use simple, kind, and empowering language in all engagements with survivors.



Chapter Five: Safeguarding and Reporting Procedures

5.1 Introduction

Safeguarding refers to the proactive and responsive measures taken to protect children, adolescents, and vulnerable individuals from harm, abuse, neglect, and exploitation. All community organisations, frontline workers, and advocates have a shared responsibility to ensure safety and to act appropriately when harm occurs or is suspected. This chapter provides clear, step-by-step procedures for preventing harm, responding to incidents, and reporting and referring cases safely and responsibly.

5.2 Types of Safeguarding Concerns

- Sexual harassment from peers, teachers, or community members (unwanted touching, comments, coercion)
- Defilement and rape of girls and minors
- Transactional sex linked to economic vulnerability (e.g., exchange for money, pads, or basic needs)
- Grooming and exploitation both offline and through digital platforms
- Abuse of power by authority figures within or around schools

Physical Abuse and Violence

- Bullying by peers, including physical fights, intimidation, and group violence
- Corporal punishment within learning institutions or at home
- Physical assault or punishment that causes harm or injury
- Violence experienced on the way to and from school, including in unsafe public spaces

Emotional and Psychological Abuse

- Verbal abuse, humiliation, and shaming by teachers, peers, or caregivers.
- Threats, intimidation, and manipulation.
- Stigma and discrimination based on gender, identity, or socio-economic status.
- Emotional neglect or isolation affecting mental wellbeing and school participation.

Neglect

- Lack of basic needs such as food, sanitary products, school fees, or safe shelter
- Absenteeism or dropout due to unmet needs or unsafe environments
- Lack of supervision and protection by teachers, exposing children to harm

Online Abuse and Exploitation

- Cyberbullying, harassment, and online shaming
- Sharing of explicit images without consent
- Online grooming and exploitation through social media platforms
- Exposure to harmful or abusive digital content

Harmful Practices and Exploitation

- Child labour interfering with education and wellbeing
- Early and forced marriages
- Sexual exploitation linked to survival or economic pressures

5.3 Recognising Signs of Abuse

Advocates should be alert to the following warning signs. It is important to observe carefully and report concerns appropriately rather than making assumptions.

Type of Sign	What to Look For
Physical Signs	Unexplained injuries, bruises, or burns; difficulty walking or sitting.
Emotional Signs	Persistent fear, anxiety, or withdrawal; sudden and unexplained changes in behaviour or mood.
Behavioural Signs	Avoidance of specific people or places; frequent absenteeism from school; sudden withdrawal or unexplained aggression.

5.4 Roles and Responsibilities

Actor	Responsibilities
Teachers	Could be the first point of contact after assault/ violence in school. Identify and report concerns, provide basic emotional support and refer for appropriate services
Community-Based Advocates	Identify and report concerns; provide basic emotional support; refer survivors to appropriate services.
Organisations (NGOs/CBOs)	Establish and enforce safeguarding policies; train staff and volunteers; ensure reporting mechanisms are in place and accessible.
Duty Bearers (Police, Children's Officers, Health Workers)	Investigate reported cases; provide statutory services; ensure the safety and welfare of those at risk.

5.5 Core Reporting Principles

All reporting must adhere to the following principles:

Principle	What to Look For
Confidentiality	Information must be shared only with relevant, authorised professionals.
Timeliness	Reports should be made immediately or within 72 hours (three days) of a disclosure or observation.
Accuracy	Record facts and direct observations only — not opinions, assumptions, or interpretations.
Survivor-Centered	Respect the survivor's wishes, safety, and dignity throughout the reporting process.

5.6 Step-by-Step Reporting Process

Step 1: Receive the Report

- Listen calmly and without interruption.
- Do not express shock, disbelief, or judgement.
- Thank the person sincerely for sharing and for trusting you.

Step 2: Ensure Immediate Safety

- Assess whether the survivor is in immediate danger.
- If necessary, move them to a safe and secure location without delay.

Step 3: Provide Basic Support

- Offer emotional reassurance and a calm, supportive presence.
- Avoid asking probing or unnecessary questions at this stage.

Step 4: Record the Information

- Document what happened, when and where it occurred, and who was involved.
- Use the survivor's own words wherever possible.
- Record only facts, do not add personal opinions or interpretations.

Step 5: Report to Appropriate Authorities

- Refer to the appropriate services: police, Children's Office, health facility, or GBV service providers.
- Use official and trusted reporting channels at all times.

Step 6: Follow Up

- Confirm that the survivor has accessed the services they were referred to.
- Continue to provide appropriate support where needed and within your role.

5.7 Do's and Don'ts in Reporting

DO	DO NOT
Report immediately or within 72 hours of a disclosure	Do not Delay making a report
Maintain strict confidentiality throughout	Do not Share information with unauthorised individuals
Use official and authorised reporting channels	Do not Attempt to investigate the case yourself
Document facts clearly and accurately	Do not Ignore signs of abuse or dismiss concerns

CASE STUDY

A community volunteer observed that a 14-year-old girl was frequently absent from school and appeared visibly distressed. Rather than ignoring the signs or confronting the situation without support, the volunteer took a careful and structured approach.

Action Taken:

- Gently and privately engaged the child in a safe and supportive environment.
- Listened carefully and learned that the child was experiencing abuse at home.
- Reported the case promptly to the local Children's Officer.
- Referred the child to the nearest health facility for assessment and support.

Outcome:

The child received the protection and professional support she needed, and the case was handled appropriately by the relevant authorities.

Key Lesson:

Early observation and timely reporting can prevent further harm and save lives.

REFLECTION ACTIVITY

Reflection Activity: Reporting Practice

Ask participants to discuss the following questions in small groups:

11. What would you do if a child reported abuse to you?
12. Who would you report to, and through which channel?
13. What challenges might you face in reporting, and how would you overcome them?

Groups share their responses and key discussion points with the wider group.

- Safeguarding protects lives, dignity, and futures.
- All advocates have a responsibility to act quickly and responsibly when concerns arise.
- Follow established reporting procedures and use official channels.
- Never attempt to investigate; refer all cases to trained professionals.
- Confidentiality and respect are non-negotiable in all safeguarding work



GBV REFERRAL & FOLLOW-UP FORM (SURVIVOR-CENTERED CASE MANAGEMENT TOOL)



GBV REFERRAL & FOLLOW-UP FORM (SURVIVOR-CENTERED CASE MANAGEMENT TOOL)

SECTION 1: CASE IDENTIFICATION (Confidential)

Case Code (for reporting):

Survivor Name (private file only):

Date of Intake:

Location of Incident:

Referred By (if applicable):

WHRD/Officer Handling Case:

SECTION 2: CONSENT & SAFETY

Informed Consent Obtained:

☐ Yes

☐ No

Consent Given For:

☐ Medical Referral

☐ Police Reporting

☐ Legal Support

☐ Psychosocial Support

☐ Shelter/Safehouse

Safe to Contact Survivor:

☐ Yes

☐ No

Preferred Method of Contact:

☐ Call

☐ SMS

☐ WhatsApp

☐ Other: _____

Safe Days/Times to Contact:

☐ Any Immediate Safety Risks Identified:

☐ Action Taken to Ensure Immediate Safety:

SECTION 3: INCIDENT SUMMARY (Objective Language Only)

(Record only essential facts. Avoid assumptions or interpretation.)

Type(s) of GBV:

☐ Physical

☐ Sexual

☐ Economic

☐ Digital

☐ Other: _____

Date/Time of Incident (if known):

Brief Description of Incident:

Relationship to Perpetrator (if disclosed):

Is Survivor a Minor?

☐ Yes

☐ No

SECTION 4: IMMEDIATE NEEDS ASSESSMENT

Medical Attention (PEP/STI care, injuries)

Psychosocial Support

Shelter/Safe Accommodation

Legal Support

Protection/Security Support

Basic Needs (food, clothing, transport)

SECTION 5: REFERRAL ACTIONS

Service Needed	Referred To Date (Facility/Organization)	Consent Confirmed (Y/N)	Notes
Medical			
Police			
Legal			
Psychosocial			
Shelter			
Other			

SECTION 7: CASE STATUS

- ☐ Ongoing
- ☐ Referred (Active Follow-Up)
- ☐ Closed
- ☐ Transitioned to Long-Term Support

SECTION 8: FINAL VERIFICATION CHECKLIST

(Tick only where applicable and confirmed with survivor consent)

- ☐ Survivor accessed medical care (e.g. Examination Test, PEP / STI treatment where needed)
- ☐ Survivor accessed psychosocial support
- ☐ Statement recorded at police station (if survivor consented - Obtain OB Number, P3 and Warrant of arrest)
- ☐ Survivor informed of legal options
- ☐ Follow-up referrals completed
- ☐ Survivor reports feeling safer than at initial intake
- ☐ Case safely closed or transitioned

IMPORTANT SAFEGUARDING NOTES

- **Case Code Use:** Case codes should be used in all reports and shared documents. Survivor names must only appear in secure, private files. This protects the survivor's identity.
- **Confidentiality:** Never share identifiable information without explicit informed consent.
- **Safe Contact:** Always confirm it is safe before contacting the survivor to avoid exposing them to further harm.
- **Objective Documentation:** Record only observable facts and direct quotes. Avoid assumptions, opinions, or investigative language.
- **Do No Harm Principle:** The survivor's safety, dignity, and choice must guide every action

FEMINIST FOR PEACE RIGHTS AND JUSTICE



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GBV CASE INTAKE FORM

STRICTLY CONFIDENTIAL

GBV CASE INTAKE FORM

STRICTLY CONFIDENTIAL

SECTION 1: INTAKE & IDENTITY

Case code: _____ (Year/Month/Number)

Date of intake: _____ Time: _____

Case Worker's name: _____

Survivor Code name: _____ Contact number: _____

Is it safe to call/text this number? ☐ Yes ☐ No

Age: _____ PWD: _____ Gender: ☐ Female ☐ Male ☐ GNC ☐ Other: _____

Emergency contact (Safe person): _____

Preferred Language of Communication: _____

SECTION 2: INCIDENT TRACKING (Basic Facts)

Type of incident:

☐ Physical Violence ☐ Sexual Violence ☐ Emotional/Psychological

☐ Economic Abuse ☐ Child Defilement ☐ Other: _____

Date & Time of Incident: _____

Location: _____

Relationship to perpetrator: _____

Perpetrator's Contacts: _____

Perpetrator's Gender: _____

Brief Description of the incident (Objective facts only):

SECTION 3: IMMEDIATE INTERVENTION

Does the survivor require immediate medical attention? ☐ Yes ☐ No

Was there any service by the victim prior? ☐ Yes ☐ No

If yes, Please state the service and the facility.

Facility _____ Service _____

Medical Referral: ☐ Yes ☐ No Facility: _____

If sexual violence, was it within 72 hours? ☐ Yes ☐ No

Police Referral: ☐ Yes ☐ No OB Number: _____

Legal Aid: ☐ Yes ☐ No Assigned To: _____

Is the survivor in a Safe location ☐ Yes ☐ No If no, Please Describe: _____

Is there an ongoing risk to the survivor's safety? ☐ Yes ☐ No If yes, please describe: _____

Safe Shelter: ☐ Yes ☐ No Location: _____

Any additional comments or observations:

SECTION 4: INFORMED CONSENT

"[] I or [] on behalf of, _____, understand that this information is being collected to assist in my care. I agree to share this data with [] Medical Services, [] Psychological Services, [] Legal Services, [] Shelter Services and relevant service providers for the purpose of my safety and recovery."

Signature/Thumbprint: _____ Date: _____

Thank You!

Thank you for your commitment to supporting survivors of GBV. Your diligence in documenting and referring cases is crucial in providing the necessary support and resources.

This process will be treated in highly confidential way by the case worker in adhering to all the terms and policies of FPRJC any breach of any information is against FPRJC policies and the case worker will be liable to FPRJC policies and the Constitution of Kenya.

Case Worker

I will adhere to all safeguarding policies in accordance to FPRJC policies of survivors' protection and Confidentiality

Name:

Phone:

Signature:

SECTION 5: FOLLOW-UP TRACKER (The Recovery Path)

Use this section to ensure the survivor is not abandoned after the first meeting.

Date	Action Taken (Call, Visit, Court Date)	Outcome/Status Update	Officer Initials

Final Verification Checklist:

- ☐ Survivor completed medical treatment (e.g., PEP/STI kit).
- ☐ Statement recorded at police station.
- ☐ Survivor feels safer than during initial intake.
- ☐ Case closed or transitioned to long-term support.

Note:

- **The case code trick:** Case Code is for the public files/reports, while the Name is for the private file. This protects the survivor's identity.
- **The safe to call box:** This is very important as it ensures that contacting the survivor doesn't put them at risk.
- **Objective language:** Stick to what can be seen or what was directly quoted.



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Have a question or need help reaching out?....



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